

JUSTIFICATION AND APPROVAL OTHER THAN FULL AND OPEN COMPETITION

1. Identification of the agency and the contracting activity.

This justification is executed by the Naval Medical Research Command (NMRC), Silver Spring, MD, on behalf of the Navy to meet Navy Expeditionary Medical Research requirements.

2. Nature and/or description of the action.

Acute undifferentiated febrile illness (AUI) is a common clinical presentation in the tropics and sub-tropic countries worldwide that often poses a diagnostic challenge and may require ongoing monitoring and evaluation among patients. As a result, vector-borne diseases such as malaria, dengue fever, Zika virus, rickettsia, and various forms of encephalitis are endemic in these areas. Limited access to healthcare and laboratory support can lead to delays in diagnosis and treatment. Moreover, infectious disease syndromes continue to impact US peacetime military operations as service members and Department of Defense (DoD) personnel travel around various geographic regions and are likely to have higher exposures to AUI than domestically in the U.S.

Here, we wanted to focus on one specific country in the Indo-Pacific region, Sri Lanka, where yearly exercises such as the Cooperation Afloat Readiness and Training (CARAT), Marine exercises (MAREX), and special forces are executed. Moreover, Sri Lanka is strategically located in the Indian Ocean coincidentally in the middle of the geopolitical competition for regional influence. All U.S. Navy vessels passing between 5th – 7th Fleets annually transit Sri Lankan waters. To date, there is no disease surveillance data available in Sri Lanka among DoD personnel for many of the febrile and vector-borne infections (FVBI) priority diseases in this region. As such, AUI diagnostics and algorithms for screening for AUI pathogens in Sri Lanka are very limited, in addition to, lack of infrastructure for extensive instrumentation and advanced technological tools. Patients with AUI are primarily managed based upon clinical case definitions, with resulting sub-optimal clinical outcomes, inadequate antimicrobial stewardship, and injudicious use of finite health resources. Surveillance and molecular epidemiology of emerging AUI pathogens in human populations and associated vectors/reservoirs is significantly lacking, which limits inclusion of appropriate laboratory diagnostics for detection/diagnosis.

The Naval Medical Research Command, Diagnostics and Surveillance Department has been selected for funding to expand our focus to other acute febrile illnesses that are prioritized in the febrile vector borne illness portfolio including dengue, chikungunya, Zika, hantaviruses, and Crimean-Congo hemorrhagic fever (CCHF). *Aedes aegypti* and *Aedes albopictus*, vector for dengue, chikungunya, and Zika virus are found throughout Sri Lanka. Dengue poses a significant public health, year-round challenge in Sri Lanka and is reported based predominantly on clinical case definition. Dengue is hyperendemic in many provinces of Sri Lanka. The highest ever recorded rates of dengue (based upon clinical case definition) were in 2017 with 186,101 cases and 440 dengue-related deaths reported to the Epidemiology Unit of the Ministry of Health. Outbreaks over the past 20 years have been associated with changing genotypes. Threats of chikungunya were not considered a public health problem in Sri Lanka until 2006 when major outbreaks were caused by the African strain of the virus, where before 2006 the Asian strain prevailed. Evidence of Zika virus in Sri Lanka was just recently reported in 2023, with the likelihood that among these infections a misdiagnosis of dengue may occur due to the similarity in clinical presentation. Laboratory confirmation is essentially needed to differentiate among these three infections for more accurate epidemiological data within the region. Rodent-borne infections, leptospirosis and hantavirus are important causes of both acute febrile illnesses, including hemorrhagic fever with renal or cardiopulmonary syndrome. Both hantavirus and leptospirosis are significantly under-reported in Sri Lanka, with ensuing morbidity and mortality. Although hantavirus infections have been reported in Sri Lanka as early as 1988, very few cases have been identified throughout various provinces due to lack of proper laboratory diagnosis. Many hantavirus cases are likely to have resulted as a misdiagnosis for

leptospirosis. To date, there have been no human cases of RT-qPCR-confirmed hantavirus infection in Sri Lanka, although Thailand-related orthohantavirus has been reported in rodents. CCHF is primary focus in area with Hyalomma ticks there is a limited presence of these ticks in Sri Lanka. However, the presence of CCHF in the Indian subcontinent raises concerns about potential introduction or shifts in the geographical range of CCHF. We will acquire new samples from the Eastern province that present with AUFI syndromes, enriched for clinically suspected cases presenting with hemorrhagic fever with renal or cardiopulmonary syndrome.

3. Description of Supplies or Services:

We need a local Sri Lankan team that is able to perform multiple tasks to complete this study:

1. To recruit a cohort of patients (n=150 to 200), 14 years old and older, hospitalized for AUFI in Eastern Province Sri Lanka and to optimally process samples for AUFI diagnostics. This cohort will be enriched for clinically suspected cases of hemorrhagic fever with renal or cardiopulmonary syndrome.
2. To collect detailed clinical, occupational, and environmental exposure data to identify associated risk factors for identified AUFI pathogens.
3. To carry out testing for AUFI pathogens (dengue, chikungunya, Zika, hantavirus, and CCHF infections utilizing species-specific molecular diagnostics, supplemented by paired serology-based assays for enhanced diagnostic sensitivity.
4. To perform genotyping on dengue and determine the strain of chikungunya virus on positive specimens.

4. Identification of the statutory exceptions permitting other than full and open competition.

The statutory exception permitting other than full and open competition for this acquisition is 10 U.S.C. 2304(c)(1) and FAR 6.302-1 sole source (including brand name) acquisitions or FAR 13.106-1(b)(1) soliciting from a single source.

Use one of the following:

10 U.S.C. 2304(c)(1), Only one responsible source.

10 U.S.C. 2304(c)(2), Unusual and compelling urgency.

10 U.S.C. 2304(c)(3), Industrial mobilization; engineering, developmental, or research capability; or expert services.

10 U.S.C. 2304(c)(4), International agreement.

10 U.S.C. 2304(c)(5), Authorized or required by statute.

10 U.S.C. 2304(c)(6), National Security.)

Be sure to include the FAR cite as well. See attached FAR explanation sheet for J&A references

FAR Part 13

FAR 13.106-1(b)(1) <\$150K

FAR 6.302-1, Only One Source (>\$150K)

FAR 6.302-2, Unusual and Compelling

5. Demonstration that the proposed contractor's unique qualifications or the nature of the acquisition requires use of the authority cited.

Dr. Neesha Rockwood is a clinician scientist at the Department of Medical Microbiology and Immunology, Faculty of Medicine, University of Colombo. During her Wellcome Trust ISSF fellowship, she was lead investigator for a study addressing the determinants of treatment outcomes in tuberculosis. She led a team that recruited and followed up to 308 patients in Khayelitsha township, South Africa. Recruitment was completed within 12 months. She also completed 2 sub-studies, including intensive all-day pharmacokinetic sampling in 100 patients and intensive sequential sampling of blood and sputum preserved for RNA extraction in 74 patients. She has a proven track record in setting up, training teams and completing both recruitment and sequential follow up of clinical cohorts, with meticulous data collection and processing of delicate samples for analyses and biobank. On returning to Sri Lanka, she has set up an AUFI study in Western and Central Province. She has the experience, qualification, and expertise to set up the cohort in Eastern Province for this study.

6. Description of efforts made to ensure that offers are solicited from as many potential sources as practicable.

Market research was performed, the results of which are in paragraph 8. This requirement will be posted to the government point of entry in accordance with FAR 5.202.

7. Determination by the Contracting Officer that the anticipated cost to the Government will be fair and reasonable.

The Contracting Officer shall determine the anticipated cost to the Government of the supplies/services covered by this Justification and Approval will be fair and reasonable by conducting a price analysis before an award is made.

8. Description of the market survey conducted and the results, or a statement of the reasons a market survey was not conducted.

Research was done by DSD member emailing and Teams Meeting to vendor to get the estimate. They also did internet searching but could not identify quality vendors.

9. Other facts supporting the use of other than full and open competition.

If other suitable vendor(s) became available, a full and open competition may be considered.

10. Listing of Sources, if any, which expressed, in writing, an interest in the acquisition.

To date, no other sources have written to express an interest. *Any sources expressing interest in response to the synopsis will receive consideration.*

11. Statement of actions, if any, the agency may take to remove or overcome any barriers to competition before any subsequent acquisition for the supplies or services required.

Future requirements will be synopsized in accordance with FAR 5.203.

12. Point of Contact:

The point of contact at the Naval Medical Research Command, Silver Spring, MD, regarding the contents of the sole source justification are the Contracting Officers in the Material Management Department (MMD) and can be reached by emailing usn.detrick.nmrc.mbx.mmd-contracts@health.mil.

**TECHNICAL AND REQUIREMENTS CERTIFICATION REQUIRED
BY NAVSUP 4200.83 series**

I certify that the facts and representations under my cognizance which are included in this justification and approval and which form a basis for this justification are complete and accurate.

Technical Cognizance

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Name	Phone	Date
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Department/Command		

Requirements Cognizance

Name	Phone	Date
Title		
Department/Command		

CONTRACTING OFFICER CERTIFICATION

I certify that this justification is accurate and complete to the best of my knowledge.

	(301) 319-9948	5/4/2026
Contracting Officer	Phone	Date